

1. Administrative & Study Identification

Full Study Title: A Study of Camizestrant in ER+/HER2- Early Breast Cancer After at Least 2 Years of Standard Adjuvant Endocrine Therapy **Short Title/Acronym:** CAMBRIA-1 **Protocol Number:** D8531C00002 **NCT Number:** NCT05774951 **Trial Status:** Recruiting **Sponsor Name:** AstraZeneca **Investigational Product:** Camizestrant (AZD9833) vs. Standard Endocrine Therapy (Tamoxifen, Anastrozole, Letrozole, or Exemestane) **Phase of Development:** Phase III **Disease Areas:** Breast Cancer, Early Breast Cancer **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess whether extended adjuvant therapy with camizestrant improves invasive breast cancer-free survival (IBCFS) compared to standard endocrine therapy in patients with ER+/HER2- early breast cancer. **Secondary Objectives:** To evaluate invasive disease-free survival (IDFS), distant relapse-free survival (DRFS), overall survival (OS), pharmacokinetics, patient-reported outcomes (Clinical Outcome Assessments - COAs), safety, and tolerability.

2.2 Background & Rationale Novel oral selective estrogen receptor degraders (SERDs) like camizestrant represent a promising therapeutic option for ER+/HER2- breast cancer. While camizestrant has shown significant efficacy over standard treatments in advanced disease settings, this study evaluates its use in the early disease setting to reduce recurrence. By introducing camizestrant as extended adjuvant therapy for patients at an intermediate or high risk of recurrence who have already completed 2 to 5 years of standard adjuvant endocrine therapy, this trial addresses the medical necessity of further reducing long-term relapse risk.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Standard Endocrine Therapy) **Blinding:** Open-label **Randomization:** Randomized (1:1)

3.2 Planned Enrollment & Duration Target \$N\$: 4,300 participants **Number of Sites:** Global multicenter (US + other countries) **Estimated Study Start:** 31-Mar-2023 **Estimated Primary Completion:** 20-Apr-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Men or women (pre- or postmenopausal) ≥ 18 years of age with ER+/HER2- early breast cancer (immunohistochemistry 0/1+/2+/in situ hybridization-negative). 2. Completed definitive locoregional therapy and standard adjuvant endocrine-based therapy (with or without CDK4/6 inhibitors) for at least 2 years and up to 5 years without disease recurrence. 3. Assessed as having an intermediate or high risk of recurrence based on defined clinical, biological, and genomic criteria.

4.2 Exclusion Criteria 1. Inoperable locally advanced or metastatic breast cancer. 2. Pathological complete response following treatment with neoadjuvant therapy. 3. Known Left Ventricular Ejection Fraction (LVEF) $< 50\%$ with heart failure NYHA Grade ≥ 2 .

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Camizestrant vs. Standard Endocrine Therapy (aromatase inhibitor or tamoxifen) $\pm$ Luteinizing hormone-releasing hormone (LHRH) agonist. **Route of Administration:** Oral **Duration of Treatment:** Planned duration of 60 months (5 years).

5.2 Concomitant & Prohibited Therapy Permitted: Prior use of CDK4/6 inhibitors; appropriate LHRH agonists depending on menopausal status. **Prohibited:** Concurrent investigational therapies or active non-protocol systemic anti-cancer treatments.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Informed Consent, Medical History, Tumor/Risk Assessment
2	0	Randomization, First Dose
3-60	Periodic	Safety Labs, Efficacy Assessments, Patient-Reported Outcomes (COAs), PK sampling
60	Month 60 / Year 10	End of Treatment

Survival and recurrence follow-up for 10 years from randomization.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Invasive breast cancer-free survival (IBCFS) based on STEEP 2.0 criteria. **Measurement Method:** Standard clinical, physical, and radiological assessments to confirm absence of invasive recurrence.

7.2 Secondary & Exploratory Endpoints * Invasive disease-free survival (IDFS) * Distant relapse-free survival (DRFS) * Overall

survival (OS) * Safety, tolerability, and health-related quality of life (COAs).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine collection via clinical visits and patient-reported outcome questionnaires. **Serious Adverse Events (SAEs):** Standard reporting timeline, typically within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) designated for trial oversight.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) Set for primary and secondary efficacy endpoints; Safety Analysis Set for evaluating AEs/SAEs. **Sample Size Justification:** Target $N = 4,300$ appropriately powered to detect statistically significant improvements in IBCFS between treatment arms. **Handling of Missing Data:** Standard imputation methods defined per the SAP for time-to-event outcomes (e.g., right-censoring).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Combination of onsite visits and remote data monitoring. **Audit Trail:** eCRF logging, tracking of treatment adherence, and resolution via data clarification forms.

11. Study Identifiers & Governance

Primary ID: D8531C00002 **Registry Number:** NCT05774951 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** CAMBRIA-1 **Official Regulatory Title:** CAMBRIA-1: A Phase III, Open-Label, Randomised Study to Assess the Efficacy and Safety of Extended Therapy With Camizestrant (AZD9833, a Next Generation, Oral Selective Estrogen Receptor Degradar) Versus Standard Endocrine Therapy (Aromatase Inhibitor or Tamoxifen) in Patients With ER+/HER2- Early Breast Cancer and an Intermediate or High Risk of Recurrence Who Have Completed Definitive Locoregional Therapy and at Least 2 Years of Standard Adjuvant Endocrine-Based Therapy Without Disease Recurrence

12. Clinical Framework (Breast Cancer Specific)

Note: Adapted from original Hyperkalemia template to match this trial's specific scope. **Primary Condition:** ER+/HER2- Early Breast Cancer **Diagnosis Threshold:** ER+/HER2- (immunohistochemistry 0/1+/2+/in situ hybridization-negative) with intermediate/high recurrence risk **Medical Methodology:** Extended Adjuvant Endocrine Therapy **Optimization Target:** Estrogen receptor degradation and prolonged invasive breast cancer-free survival

13. Study Procedures & Methodology

Management Pathway: Extended adjuvant clinical pathway replacing or extending initial 2-5 year standard ET **Education Protocol (HCP):** Protocol-specific safety training and tumor assessment guidelines **Education Protocol (Patient):** Informed consent, reporting of patient-reported outcomes (COAs), and long-term oral adherence **Quality Audit Mechanism:** Clinical Outcome Assessments (COA) compliance tracking and routine monitoring

14. Observation & Follow-Up Schedule

Total Duration: 60 months of treatment with a 10-year total follow-up **Onsite Visit Cadence:** Periodic protocol-defined milestones throughout the 60-month duration **Remote Monitoring Frequency:** Supplemental to in-person clinical safety checks **Checklist Review Interval:** Regular intervals mapped to standard-of-care oncology visits

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				